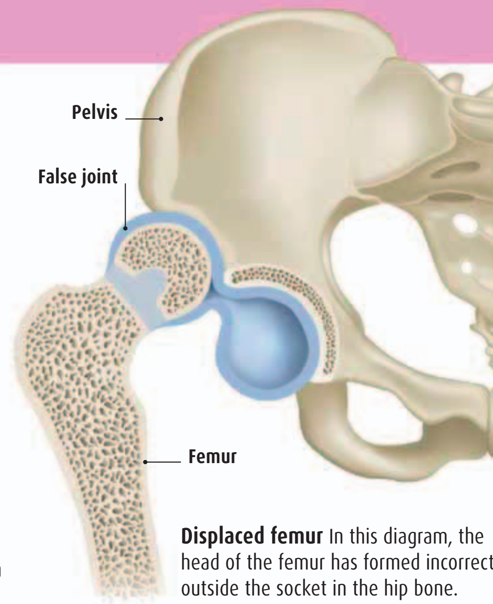
Hip dysplasia

» What is this?
The long bone of the thigh (the femur) connects to the pelvis in a ball-and-socket joint at the hip, which gives us full range of circular movement in our legs. In about 2 percent of births, babies are born with femur joints (one or both) that completely or partially malform outside the socket. Your baby's hips will be tested for this condition within three days of birth—a doctor or doctor will manipulate her upper thighs in the hip sockets to check for freedom of movement. The condition tends to run in families and is more common in baby girls than in boys.

» What can be done?
Sometimes it's just a matter of ligaments tightening up around the joint to hold the femur bone in place, which can happen naturally. If your baby has hip dysplasia caused by malformation in the bone, however, she may need to wear a special harness to hold her legs in position so that as the bones of the pelvis harden, the socket forms properly around the top of the femur bone. If hip dysplasia isn't picked up until after your baby is six months old, or if treatment with the harness hasn't worked, your baby will need surgery to position the femur correctly within the socket. After surgery, your baby will need to wear a cast until the bones have reformed around the corrected femur, a process that usually takes about three months.



Displaced femur In this diagram, the head of the femur has formed incorrectly outside the socket in the hip bone.

Undescended testes

» What is this?
During the third trimester of pregnancy, a baby boy's testes should descend into the scrotal sac. However, in up to 4 percent of baby boys, the testes fail to descend and the baby is born with an empty scrotum. This doesn't cause any pain.

» What can be done?
Most of the time, the testes will descend without any intervention before the baby is six months old. If they don't, your baby will need to have surgery to move them into the correct position. The surgery usually takes place before he reaches the age of five.

Talipes

» What is this?
More commonly known as club foot, this occurs when the tendons on the inside of a baby's lower leg are tighter and shorter than those on the outside. The result is that the foot (or feet—about half of all cases of talipes show up in both feet) turns inward. Usually, the bones of the ankle are also poorly formed. It's more common in boys, and occurs in about one in 1,000 babies. The condition runs in families. There are several types of club foot, ranging from isolated conditions that can be easily corrected to types that are an indication of other, chromosomal abnormalities.

» What can be done?
Soon after birth, your baby will begin a weekly program of manipulation and casting to coax the foot into the correct position as the bones grow. Once this process is over, he may need a simple surgery to release the tension in his Achilles tendon. To prevent retightening in the tendon, he will probably have to wear special harnesses on his feet continually, and then only nightly, until he is four or five years old. After this, children usually recover fully.

Syndactyly

» What is this?
Your baby's hands and feet form in the early weeks of pregnancy. At about five weeks, they look like paddles, with fingers or toes joined together. They begin to separate about two to three weeks later. Syndactyly occurs when two or more of the digits of the hands or feet fail to separate properly, leading to a webbed appearance. The effect may be superficial, in just the skin, or the bones may have fused. It affects about one in 2,000–3,000 babies, and can run in families.

» What can be done?
Your baby will need surgery to separate the bones and/or skin of the fingers or toes that are joined together. If she has more than two conjoined digits, she will need more than one surgery, because only one side of each digit can be operated on at a time in order to preserve the blood flow to the whole hand or foot.

Tongue-tie

» What is this?
Tongue-tie occurs when the frenulum, the small piece of tissue that lies beneath the tongue, connecting it to the bottom of the mouth, is too far forward, so the tongue can't move forward or from side to side effectively. This can make breast-feeding especially difficult and can sometimes cause sore and ulcerated nipples for the mother if the baby is unable to achieve a good latch. If your baby is feeding well, you may never notice that she is tongue-tied. However, if you are having problems feeding, perhaps finding that your baby doesn't latch on to your nipple properly, tongue-tie could be the cause. Tongue-tie affects 4–10 percent of newborn babies and is more common in boys than girls.

» What can be done?
If your baby is having problems with feeding and it's thought that tongue-tie is the problem, doctors may perform a tongue-tie division, a procedure that involves a painless snip in the tightened frenulum to enable the free movement of the baby's mouth and tongue.

Retinopathy of prematurity (ROP)

» What is this?
Images are formed on the back of the eye, at the retina. Babies born before 31 weeks are at risk of developing abnormal blood vessels leading to the retina, impairing its ability to receive information and so distorting vision. It occurs in about 20 percent of premature births. There are no outward signs of the condition and diagnosis relies upon assessment by an ophthalmologist at birth.

» What can be done?
After birth, an eye doctor will shine a flashlight into your baby's eyes to assess the health of her retinas. Your baby may have drops put into her eyes to give the doctors a better picture of what's going on. Laser surgery may prevent the advancement of the condition and save sight.

» What does it mean for my baby?
Mild forms of ROP, which are the most common, will correct themselves in time, as the blood capillaries feeding the retinas settle down. More severe forms may lead to the retina detaching from the layers beneath it; laser surgery may correct this but it isn't always successful, and often babies will have at least some form of visual impairment, particularly in their peripheral vision.